

February 23, 2026

Submitted Electronically

Robert F. Kennedy, Jr.,
Secretary
U.S. Department of Health and Human Services
200 Independence Avenue SW
Washington, DC 20201

Re: HHS Health Sector AI RFI; RIN 0955-AA13

Dear Secretary Kennedy:

Oracle Health & Life Sciences. (“Oracle Health”) appreciates this opportunity to comment on the U.S. Department of Health and Human Services (the “Department”) Request for Information¹ (“RFI”) on Accelerating the Adoption and Use of Artificial Intelligence (“AI”) as Part of Clinical Care. Oracle Health supports the Administration’s efforts to establish a forward-leaning, industry-supportive, and secure approach to accelerate the adoption and use of AI as part of clinical care.² Oracle Health further supports the rapid adoption and use of AI in clinical care, to foster public trust and confidence in modern technology solutions, to reduce uncertainty that impedes AI innovation, and to align federal incentives so that AI is deployed in ways that enhance productivity, reduce burden, lower healthcare costs, and improve health outcomes for patients, caregivers, and communities.³

Oracle Health shares the Department’s commitment to U.S. leadership on AI and provides the following recommendations based on its leading role in the healthcare AI ecosystem.

- The Department can streamline administrative hurdles to private sector innovation in AI for healthcare by limiting regulation that might stifle innovation and ensuring alignment of any regulations that impact AI in healthcare, prioritizing the development of publicly available benchmark data sets for AI model evaluation, and updating standards for patient data privacy and deidentification to incorporate AI-relevant provisions.
- The Department can help to align payment incentives with the adoption and use of AI as part of clinical care to support clinician buy-in and improve clinical outcomes, particularly for clinicians and patients in rural communities.
- The Department can clarify that, while AI is a revolutionary technology that is actively transforming healthcare delivery in the U.S., there is a strong legal framework and existing industry standards that support the introduction of innovative technology in

¹ 90 Fed. Reg. 60108 (Dec. 23, 2025).

² *Id.*

³ *Id.*

healthcare. Guidance by the Department can help healthcare stakeholders to understand how these existing legal frameworks can apply to the safe adoption and use of AI as part of clinical care.

The comments below provide further detail on each of these recommendations.

I. Ways to Enhance Private Sector Innovation in AI for Healthcare and its Adoption and Use in Clinical Care

Healthcare AI innovation has been developing under current frameworks; however, the Department can help address areas of friction to private sector innovation in AI for healthcare and its adoption and use in clinical care, including regulatory alignment, data availability for testing, and clear standards for patient data privacy and deidentification. Below, we outline these issues and provide suggestions for how the Department can help to address them to better promote private sector innovation.

a. Regulatory Alignment

Oracle Health applauds the Department's efforts to "establish a regulatory posture on AI that is well understood, predictable, and proportionate to any risks presented to enable rapid innovation while protecting patients and the confidentiality of their identifiable health information and maintaining public trust."⁴ A coordinated federal approach is essential to reducing fragmentation and compliance burden across the healthcare ecosystem, and Oracle Health appreciates the opportunity to comment on how the Department may develop such an approach.

Currently, innovation in AI for healthcare is slowed by confusing and sometimes conflicting rules and guidance on AI across major federal programs and agencies. As an example, for nearly two decades, stakeholders from the medical community have raised concern that overly burdensome U.S. Food and Drug Administration ("FDA") regulation of software as a medical device could stifle innovation.⁵ These stakeholders have also expressed concern about potentially duplicative and inconsistent regulatory regimes from the Assistant Secretary of Technology Policy/Office of the National Coordinator for Health Information Technology ("ASTP/ONC"), FDA, and other federal agencies. This concern led to the 21st Century Cures Act in 2016 ("Cures Act"), which officially carved out certain software from the FDA's device jurisdiction and adopted a pro-innovation approach to software regulation.⁶

In 2022, the FDA issued Clinical Decision Support ("CDS") Software Guidance for Industry and Food and Drug Administration Staff (the "2022 CDS Guidance") which is inconsistent with the

⁴ *Id.*

⁵ See, e.g., Brian Dolan, *In tense hearing, congressman declares "software is not a medical device"*, MOBI HEALTH NEWS (Nov. 21, 2013), <https://www.mobihealthnews.com/news/tense-hearing-congressman-declares-software-not-medical-device>.

⁶ 21st Century Cures Act, Pub. L. 114-255 § 3060, 130 Stat. 1033, 1130-32 (2016) (codified at 21 U.S.C. § 360 note) ("Cures Act").

risk-based approach outlined in the Cures Act.⁷ In the 2022 CDS Guidance, FDA introduced brand new factors that do not exist in the statute or even the previous drafts of the CDS guidance, such as “time-critical decision-making” and “automation bias,” that significantly expand FDA’s jurisdiction. The 2022 CDS Guidance also creates a presumption that risk scores or other CDS offering only one treatment or diagnostic recommendation would be treated as a device, regardless of the information provided to a physician to explain that recommendation.

In January 2026, the FDA updated the 2022 CDS Guidance (the “2026 CDS Guidance”) to indicate its intent to exercise enforcement discretion toward a CDS software function that provides a single output or recommendation in scenarios where only a single option is “clinically appropriate”.⁸ Although the 2026 CDS Guidance is slightly more aligned with the regulatory framework created by the Cures Act, ultimately the FDA’s current posture remains at odds with the risk-based approach in the Cures Act and continues to stifle innovation. Most notably, FDA continues to treat CDS software functions that provide clinically appropriate recommendations in time critical conditions as device functions requiring premarket authorization, despite that time criticality and automation bias are not concepts recognized in the statute.

The implications of the FDA’s overreach are significant. The technology exists today to improve outcomes for patients and ease burdens for healthcare providers, but electronic health record (“EHR”) companies such as Oracle Health are hesitant to put these services into use for fear of incurring massive regulatory requirements. It is past time to restore a pro-innovation approach to clinical decision support.

Moreover, FDA’s recent guidance says nothing to clarify when an EHR or other software vendor will be held responsible for the software that healthcare institutions such as hospitals may create from the software tools they are provided by vendors. Healthcare institutions increasingly want to customize their clinical decision support to the needs of the populations they serve. AI platforms and software toolkits can now allow for provider specific innovations and solutions. Where a provider supplies an AI platform, toolkit, or development environment to create rules-based clinical decision support prompts, and providers customize those tools for the needs of their institution, the software vendor has supplied general-purpose software and should not be treated as a medical device developer for the tools that the healthcare institutions create and customize.

Oracle Health urges the Department to develop guidance that: (i) aligns to the statutory text of the 21st Century Cures Act; (ii) recognizes that many additional CDS software functions that may meet the definition of a medical device do not, under a risk-based framework, require FDA oversight; (iii) clarifies that healthcare institutions that configure their EHRs to include CDS that supports their practices as an institution are not subject to the FDA’s device requirements; and (iv) clarifies that software developers that create those CDS tools at the specifications of healthcare institutions are not subject to device requirements.

⁷ FDA, Guidance for Industry and FDA Staff, *Clinical Decision Support Software* (Sept. 2022), <https://www.fda.gov/media/109618/download>.

⁸ FDA, Guidance for Industry and FDA Staff, *Clinical Decision Support Software* (Jan. 2026), <https://www.fda.gov/media/109618/download>.

More broadly, the Department should work to align AI-related policies and guidance across major federal programs and agencies to create a more consistent environment for AI innovation and adoption. Coordination among the relevant agencies and regulatory bodies can help harmonize expectations for oversight, reimbursement, quality measurement, and clinical implementation. Oracle Health supports the Department’s initiatives to further regulatory alignment and welcomes the opportunity to participate further in such projects.

b. Access to Clinically Relevant Benchmark Datasets

While Oracle Health maintains access to high-quality de-identified data to support development and validation of its AI systems, there remains a need for clinically relevant, standardized evaluation benchmarks that enable consistent, pre-deployment testing and transparent comparison across tools. Oracle Health encourages HHS to prioritize development of privacy-preserving benchmark evaluation datasets and test frameworks (e.g., disease-specific evaluation sets) intended for validation and comparability, not as general-purpose training datasets, with appropriate governance (e.g., secure access, auditing, or “models-to-data” approaches). Disease-specific evaluation sets, longitudinal datasets, and representative cohorts would enable more transparent comparison across tools and reduce variability in how performance is addressed.

c. Standards for Patient Data Privacy and Deidentification

Patient data, when aggregated and deidentified, allows for deep research into patient and population health. In the age of AI, deidentified data can still pose a risk to patient privacy. To mitigate this risk, the deidentification standard can be modified to make data more useful once it has been deidentified while still preventing reidentification. Furthermore, the Department can develop policy to enable the use of privacy-enhancing technologies.

Oracle Health would welcome the opportunity to work with the Department to modernize the Health Insurance Portability and Accountability Act of 1996 (“HIPAA”) Privacy Rule policies and deidentification standards. The HIPAA Privacy Rule was designed well before the development of AI technology and did not sufficiently contemplate how best to protect health data while also leveraging the incredible learnings that such data can provide to improve care for population health. Modifications or clarifications to HIPAA deidentification policies and considering alternatives, including leveraging privacy-enhancing technology, can provide greater access to data to permit model training while better protecting patient health data in this new era of technology.

II. Incentivizing the Effective Use of AI in Clinical Care

Oracle Health applauds the Department’s focus on payment policy changes to promote access to high-value AI clinical interventions, foster competition among clinical care AI tool builders, and accelerate access to and affordability of AI tools for clinical care.⁹ In connection with such efforts, Oracle Health encourages the Department to recognize the investment of time and

⁹ 90 Fed. Reg. 60108 (Dec. 23, 2025).

training necessary to adopt and implement AI-enabled technologies. While many AI technologies are familiar and user-friendly, others can take time for clinicians to evaluate, learn to use, and integrate into care delivery. It also comes at an expense.

Oracle supports the Department's efforts to incentivize the effective use of AI in clinical care and encourages the Department to develop and deploy payment or reimbursement models which recognize clinician time associated with testing and learning to use and implement AI-enabled technologies and with lost revenue that could come from adopting technology that improves outcomes but reduces clinician time. The federal government provided financial incentives for adoption and meaningful use of EHRs. This had a substantial and positive effect on adoption. Similarly, it is critical that the Department consider payment and reimbursement models that can support adoption of technology that can reduce errors, improve access to care, and increase efficiency.

In that same vein, it is important for the Department to use its position to help educate the healthcare provider community about use of AI in healthcare delivery. Specifically, increasing general acumen for the how AI works and the distinctions between general purpose AI that individuals may use in their daily lives versus purpose-built health AI systems and models that are incorporated into EHRs and other health applications. Healthcare providers are inundated with messages about AI in healthcare and there is a need for education from a trusted, reliable source to dispel misconceptions and expand knowledge to help encourage and drive adoption.

In tandem with the Rural Health Transformation Program ("RHTP") authorized by the One Big Beautiful Bill Act,¹⁰ Oracle Health urges the Department to ensure that AI-enabled technology (including infrastructure technology such as AI-enabled EHRs) is available and affordable to providers in rural communities. The RHTP's Strategic Goals include sustainable access to care, innovative care, and tech innovation; AI-enabled technologies can assist clinicians and recipients of RHTP funds achieve each of these goals.¹¹ In fact, AI-enabled technologies promise to disproportionately help rural providers to improve care access, quality, and affordability, by increasing provider productivity and reducing provider burden while improving patient outcomes.¹² Oracle Health encourages the Department, and the ultimate recipients of RHTP funds, to ensure that the adoption of AI for clinical care is a key component of RHTP implementation plans.

III. Legal Framework Considerations

The history of technological innovation and adoption in healthcare has been transformative, from the discovery of penicillin to the invention of advanced imaging to the development of modern-day EHRs. While each of these technologies represents an enormous advancement in science and

¹⁰ One Big Beautiful Bill Act, Pub. L. 119–21 § 71401, 139 Stat. 327 (2025).

¹¹ CMS, *Rural Health Transformation Program* (Dec. 2025), <https://www.cms.gov/priorities/rural-health-transformation-rht-program/overview>.

¹² See American Hospital Association, *Rural Hospitals and the AI Advantage: Turning Constraints into Catalysts* (Jan. 2026), <https://www.aha.org/aha-center-health-innovation-market-scan/2026-01-26-rural-hospitals-and-ai-advantage-turning-constraints-catalysts>; National Consortium of Telehealth Resource Centers, *AI in Rural Health* (Dec. 2024), <https://telehealthresourcecenter.org/wp-content/uploads/2024/12/AI-in-Rural-Health-Final.pdf>.

medicine, the legal framework in the U.S. regarding their implementation and use has remained largely the same: at the end of the day, the clinicians and the health systems they work in retain ultimate decision-making authority for how they treat patients, and the providers are given discretion to utilize technologies that may help them in ensuring a positive patient outcome.

While AI is undoubtedly another such revolutionary technology, Oracle Health encourages the Department to view AI-enabled technology that augments clinical care simply as another tool for providers to use in their workflows and in their professional discretion. While AI-enabled technologies have the potential to improve patient diagnosis, treatment and outcomes and reduce clinician burdens, decisions about how to treat patients still rests with clinicians and the health systems they work in. For example, AI-enabled technologies such as CDS can improve care quality and outcomes by quickly delivering actionable data for clinicians to consider in patient treatment plans. However, ultimately health systems and other healthcare institutions develop practice guidelines for their clinicians to follow, and AI-enabled technologies are only intended to augment, not supplant, those practice guidelines and a clinician's professional decision-making and judgment. At all times, healthcare providers remain in control of patient care decisions.

To further empower providers in their adoption and use of AI for clinical care, Oracle Health encourages the Department to support clinician education and training in AI through educational grant programs and aligned payment and reimbursement models. Similarly, Oracle Health encourages state professional licensing boards to incorporate AI-enabled technology training into professional continuing education requirements and would support Department-led efforts to incentivize the same. Oracle Health also urges health systems and other providers to ensure they have AI governance structures in place and regularly review industry guidance and best practices on the responsible use of AI in healthcare.

For developers of AI-enabled technologies, existing industry standards and guidance already address many of the questions raised by such technologies. For example, Oracle Health is certified to the ISO 42001 standard regarding AI management systems and supports ASTP/ONC's decision support interventions certification and related industry standards and certifications. There are also the NIST AI Risk Management Framework and the HIMSS AI policy principles. In light of the rapid pace of AI innovation, Oracle Health encourages the Department to defer to industry standards and practices which have already proven more than capable of ensuring safety, quality, and accessibility in AI innovation and use any levers to encourage new innovators to comply with these industry standards.

Therefore, while slight modifications to existing legal frameworks may be helpful to ensure that AI-enabled technologies only augment and do not replace clinical decision-making, Oracle Health encourages the Department to treat AI-enabled technologies similarly to formerly transformative technologies – simply another tool at a clinician's disposal to improve their treatment of patients.

IV. Conclusion

Oracle Health supports the Department's efforts to establish a forward-leaning, industry-supportive, and secure approach to accelerate the adoption and use of AI as part of clinical care. In furtherance of these goals, Oracle Health encourages the Department to: (i) align regulation of AI in healthcare, prioritize the development of publicly available benchmark data sets for AI model validation and evaluation, and update standards for patient data privacy and deidentification to incorporate AI-relevant provisions; (ii) align payment incentives with the adoption and use of AI as part of clinical care to support clinician buy-in and improve clinical outcomes, particularly for rural healthcare providers; and (iii) work within existing legal frameworks and encourage alignment with industry standards to address responsible development and adoption and use of AI as part of clinical care.

Oracle Health values the opportunity to comment on the topics raised in the RFI. If you have any questions or if Oracle Health can provide any additional information or clarification on these comments, please do not hesitate to contact me directly at 410-925-7904 or alex.mugge@oracle.com.

Sincerely,

A handwritten signature in black ink, appearing to read 'Alex Mugge', with a stylized, cursive script.

Alex Mugge
Senior Director, Global Health Policy and
Regulatory Intelligence